DIABETIC RETINOPATHY DIAGNOSTIC REPORT

Automated IVP & Deep Learning Clinical Advisory System

Patient Name: John Doe

Scan Date: 2026-09-03

Age / Sex: 52 / Male

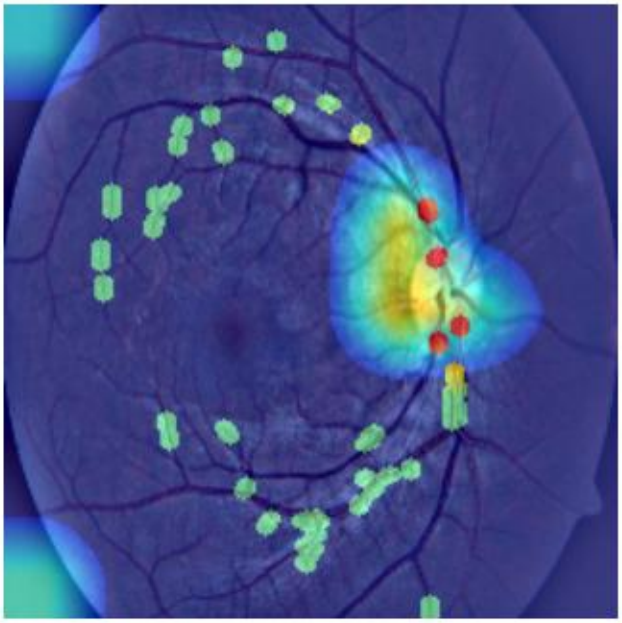
Examined Eye: Right Eye (OD)

DIAGNOSIS: Grade 3: Severe Non-Proliferative DR (NPDR)

Confidence: 97.6%



Raw Retinal Fundus Scan



Grad-CAM Lesion Heatmap

CLINICAL RECOMMENDATIONS & TREATMENT PLAN

▯ Follow-up Schedule: Urgent retina specialist consultation within 2 to 4 weeks.

▯ Glycemic & Systemic Target: Strict metabolic control; avoid rapid drops in blood sugar without specialist oversight.

Key Clinical Actions:

- URGENT referral to Retina Specialist within 2▯4 weeks.
- Perform OCT and Fundus Fluorescein Angiography (FFA) to map capillary non-perfusion.
- Prepare for prophylactic laser treatment or anti-VEGF therapy before progression to proliferative phase.
- Close patient surveillance for neovascularization signs.

Medication / Laser Guidance: Panretinal Photocoagulation (PRP) evaluation or intraocular Anti-VEGF injections indicated to reduce ischemia.